

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 26

Age: 49 years old | Sex: Female

MRN: 000-TEST-26 | Admission Date: 06/10/2026 | Discharge Date: 06/13/2026

Facility: Metro Medical Center, General Medicine Unit

****PRIMARY DIAGNOSIS & COMORBIDITIES****

Primary: Type 2 Diabetes Mellitus with acute hyperglycemic episode

Secondary diagnoses:

- Hypothyroidism on replacement therapy
- Generalized Anxiety Disorder
- Hypertension, controlled

****HOSPITAL COURSE****

Patient 26 presented to the ED on 06/10/2026 with polyuria, polydipsia, and fatigue × 3 days. Admission labs revealed serum glucose 487 mg/dL, pH 7.31 (mild metabolic acidosis), HbA1c 9.2%, and adequate renal function (Cr 0.9). No evidence of diabetic ketoacidosis or hyperosmolar state. Patient had been non-compliant with home metformin 1000 mg BID regimen for approximately 2 weeks due to GI upset.

During admission, patient received IV fluid resuscitation, initiation of basal-bolus insulin therapy, and diabetes education by certified diabetes educator. Blood glucose values

normalized by hospital day 2 (range 140–220 mg/dL). Endocrinology consultation performed 06/11/2026; recommended insulin initiation as primary therapy given HbA1c and hyperglycemic presentation. Thyroid-stimulating hormone (TSH) reviewed—currently stable on levothyroxine 75 mcg q.d. Anxiety managed with standing sertraline; no acute episode during stay. Patient discharged in stable condition with improved glycemic control and education reinforcement.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
Insulin glargine (basal)	20 units	SC q.h.s.	Ongoing
Insulin aspart (bolus)	4–8 units	SC w/ meals TID	Ongoing
Metformin XR	1000 mg	PO q.d. evening	Ongoing
Levothyroxine	75 mcg	PO q.d. morning	Ongoing
Sertraline	50 mg	PO q.d.	Ongoing
Lisinopril	20 mg	PO q.d.	Ongoing
Atorvastatin	40 mg	PO q.h.s.	Ongoing

****GLUCOSE MONITORING & MANAGEMENT****

- Check fingerstick capillary glucose before meals, at bedtime, and once weekly at 3 AM (fasting)
- Document all readings in patient logbook; bring to all follow-up appointments
- Target range: 100–180 mg/dL
- Report readings consistently >250 mg/dL or <70 mg/dL to provider immediately

****FOLLOW-UP APPOINTMENTS****

- Endocrinology clinic: 06/27/2026 (2 weeks post-discharge) — insulin titration, A1c reassessment in 3 months
- Primary care physician: 06/20/2026 — medication review, TSH recheck
- Diabetes education refresher: Arrange with RN educator within 1 week

****ACTIVITY & WEIGHT-BEARING****

- No restrictions; encourage daily ambulation (goal 30 min most days)
- Avoid strenuous exertion until cleared by PCP

****DIETARY RECOMMENDATIONS****

- Consistent carbohydrate intake at